

# Ear Injury - Male (Adult)

Algorithm 1285 | Adult (18 years and older) | Gender at birth: Male

## Definition

- Ear trauma assessment, generalized from NHS.UK's earache guidance's hearing-loss and discharge red flags applied to injury.
- Adult patient.
- Male.

## Initial assessment

1. [OPEN\_TEXT] How did the ear get injured?
2. [YES\_NO] Any bleeding, discharge, or sudden hearing change?
3. [DURATION] When did it happen?
4. [OPEN\_TEXT] Record age, head-injury symptoms, anticoagulants or bleeding disorder, pregnancy, immune suppression, object or blast exposure, and whether the mechanism or caregiver history raises safeguarding concern.
5. [YES\_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?

## Triage and redirect questions

### **1. Redirect → Ear - Foreign Body**

Is the main concern an object lodged in the ear rather than an injury to the ear?

An object lodged in the ear belongs in the Ear - Foreign Body pathway.

#### **1. Disposition level 100**

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

#### **2. Disposition level 78**

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

#### **3. Disposition level 100**

Is there reduced consciousness, seizure, repeated vomiting, severe headache, clear fluid or blood from the ear after head injury, sudden hearing loss, severe dizziness or imbalance, facial weakness, major bleeding, penetrating trauma, or an embedded object?

Clear fluid or blood from the ear after trauma can indicate a skull base fracture; consistent with NHS.UK earache guidance's discharge/hearing-loss red flags applied to a trauma context.

#### **3. Disposition level 70**

Is the outer ear swollen, bruised, cut, or increasingly painful?

Blunt trauma to the outer ear can cause a hematoma that needs prompt drainage to prevent permanent deformity ("cauliflower ear").

#### **3. Disposition level 15**

Is this a minor bump to the ear with none of the features above?

A minor ear bump without bleeding, discharge, or hearing change is manageable at home.

## Advice

### **Immediate child safeguarding danger**

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

### **Urgent private child safeguarding assessment**

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact

reporting, consent, documentation, and information-sharing rules are local approval required.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

### ***Emergency ear and head-injury precautions***

- Call Qatar 999 now. Do not plug, pack, irrigate, put drops into, or remove an embedded object from the ear. Keep the head and neck still, allow drainage onto a loose clean dressing without pressure, give nothing by mouth if alertness is impaired, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening dizziness, vomiting, seizure, facial weakness, or decreasing consciousness; increasing bleeding or clear drainage.

### ***Prompt in-person ear injury review***

- Use the Qatar governance-approved in-person ENT or emergency pathway. A wrapped cool compress may be placed over an intact outer ear without pressure; do not drain swelling, probe the canal, or use drops. Address pediatric or vulnerable-person safeguarding privately under local policy.
- Call Back If: hearing change, dizziness, discharge, facial weakness, increasing swelling, or pain.

### ***Care for a clearly minor outer-ear bump***

- Use a wrapped cool compress without pressure and confirm age- and pregnancy-appropriate pain medicine with a pharmacist. Keep the canal dry and do not insert anything into it.
- Call Back If: bleeding, discharge, hearing change, dizziness, swelling, or worsening pain.

## **First aid**

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar 999 now. Do not plug, pack, irrigate, put drops into, or remove an embedded object from the ear. Keep the head and neck still, allow drainage onto a loose clean dressing without pressure, give nothing by mouth if alertness is impaired, and do not allow self-driving. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## **References**

- NHS.UK, "Earache" (discharge/hearing-loss red flags), <https://www.nhs.uk/conditions/earache/> (page last reviewed 27 October 2025) - generalized to ear trauma

## **Provenance**

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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